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Comments on CMS-0042-NC, RIN 0938-AV68

Thank you for the opportunity to respond to the Health Technology Ecosystem RFI – RIN 0938-AV68, which shows the commitment of the Centers for Medicare and Medicaid Services (CMS) to improve the healthcare system. At Claritev, we believe that transparency can drive cost containment and quality improvement. We also believe that delivering actionable data to the point of service or payment in a timely fashion is a key element of how we can improve workflows and results across our health care ecosystem. By reducing abrasion and enhancing consumer experience, while addressing provider concerns and administrative burdens, payors of all types can maximize the value of every health care dollar spent.

For the past 40 plus years, Claritev has advanced this cause through helping to optimize payments, creating value-based networks, harnessing the power of data analytics to drive better payment integrity, and facilitating quality outcomes and affordability across the health care ecosystem. We support providers in similar ways by creating pathways to better payment, improving system-wide performance, and helping to manage risk through data-driven interventions. Using data insights and automated tools, Claritev also helps patients and caregivers navigate the complexities of the system.

In the new world of AI-driven workflows, we have a commitment to ensure Claritev uses AI in a way that's safe, secure, ethical and transparent for the benefit of our clients and that of consumers at large. We have joined the Coalition for Health AI (CHAI), which is committed to advancing the responsible development, deployment and oversight of AI in healthcare. This coalition includes industry, government, academia and patient communities to ensure that all views are represented, and all concerns are addressed.

Claritev is providing direct responses to several questions posed in this RFI and is also offering our perspective on multiple overarching themes and considerations that, if applied across CMS, can:

- Enhance the existing ecosystem to optimize the patient healthcare journey in conjunction with stakeholders, patients and caregivers
- Empower beneficiaries with better access to tools, personalized care navigation and information about treatments and cost so that beneficiaries can make informed, value-driven care decisions
- Reduce systemic barriers to care by expanding access in underserved populations and closing provider gaps in rural areas
- Utilize proven and emerging AI-based solutions to improve patient and caregiver experiences by leveraging data across multiple platforms to build a patient-centric experience
- Incentivize providers to optimize care with real-time, accurate and actionable data
- Aggressively modernize tools to quickly identify and mitigate fraud, waste and abuse through AI.

We look forward to further discussions on all of our recommendations and perspectives.

General Recommendations

As a country, we have made tremendous progress toward digitizing health data, addressing interoperability and creating direct access to electronic health records for consumers, providers and payors to work together to improve affordability, address risks and coordinate care. We are now able to better manage the health care journeys of consumers through every encounter, and deliver better data to help manage population health, chronic disease conditions and emergent health care challenges.

We are on the cusp of creating a nationally integrated data ecosystem, with standard rules of the road, encouraging all players across the health care sector to exchange data appropriately to deliver better outcomes for patients and better value for the dollars we spend. While the TEFCA/ 21st Century Cures Act is already focused on improving clinical data exchange, its next building block centers around bringing payors data to leverage clinical and administrative data to improve processes like prior authorization. We need to accelerate that process so that clinical and administrative patient data can be curated through a single exchange infrastructure, allowing patients and providers alike to have the most informed and actionable data to enhance the navigation of both the clinical and administrative burdens of our healthcare systems. The types of data should include, but are not limited to known conditions, hospitalizations, outpatient facility visits, claims history, eligibility and benefits information, general encounter summary data, prescriptions, immunizations, medication (including history) and current care teams.

We believe that removing data-sharing barriers, reducing the monetization of certain data and encouraging the transmittal, curation and delivery of relevant data to the point of service or payment would benefit patients, caregivers, providers and payors alike. Building upon the current Trusted Exchange Framework and Common Agreement infrastructure (TEFCA), more participants and data sources can be added using APIs based on the Fast Healthcare Interoperability Resources (FHIR) standard. This will allow the connection of existing systems and new data sources that follow current common standards and interoperability requirements. Along with other state and federal efforts, we have the building blocks in place to curate this holistic view of patients and providers, focusing on results and outcomes and encouraging best practices in an impactful manner to improve administrative burden, the patient experience and care received across the country.

Importantly, an exchange of a more comprehensive data set will facilitate real-time access to accurate information across the continuum of care. Such a system could also curate anonymized data on a secure, scalable architecture with role-based access to protect PHI and PII, while also allowing for appropriate population health research and risk management on a grand scale. Early intervention, best practice dissemination, targeted health care initiatives based on the needs of underserved populations — these

would all be outcomes readily available through such an integrated infrastructure. Ultimately, payors and providers alike would deliver accurate data insights, resulting in lower overall healthcare costs for consumers and for the system as a whole.

With the 21st Century Cures Act, payors already have upcoming interoperability requirements, and building upon that, once we create this virtual, fully integrated data exchange to access patient data across multiple EHRs and independent medical systems, it can present patients and their caregivers with actionable insights about patients' conditions, available and recommended therapies, diagnostics, clinical and physician appointments and pricing based on benefit plans in the provider marketplace.

This information can be standardized and continually enhanced using AI and machine learning to improve recommendations for patients and caregivers, guiding them to quality healthcare and supportive care including assistance with navigating the Medicare, Medicare Advantage and Medicaid systems to improve patient outcomes.

There are hundreds of analysis and data and decision tools in the market that can assist patients, caregivers, plans and providers alike. Developing and testing these tools in a national healthcare center of data and innovation excellence could help drive solutions into the market faster. These tools can transform data into actionable and timely insights that can improve short- and long-term outcomes.

Select responses to CMS questions based on Claritev's expertise

Section B: Patients and Caregivers

PC-9: What additional CMS data sources do developers view as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers?

When selecting care and understanding options, price transparency is at the top of the list. Patients and their caregivers need to understand how much treatment will cost for that provider, for that service, at their current benefit levels within their current plan. Additionally, patients should also have access to quality ratings of the providers they access so that they can make the care choice that suits them best given their circumstances.

Finally, any data elements should support the concept of personalized insights for patients. By creating better tools with better data, patients and their caregivers can be empowered to make informed and value-driven care decisions and remove friction in the patient experience.

Data including appointment reminders, medication refill reminders, test results (translated from medical jargon into easily digested summaries), and instructions about care are all critical examples of personalized insights. Imagine a patient history timeline with milestones that you can click on to display information about their care team, encounter summaries, recommended care pathways based on current coverage and quality scores, information about referrals and follow-up appointments and therapies. All of this would provide personalized insights for individual patients. It would also:

- Provide a clear picture of the patient's history, status and potential next steps through automatic reminders for appointments, medication refills, review of test results and analysis of data from medical devices
- Better align coverage with managing chronic conditions, staying current with preventative care and healthy visits, and identifying and accessing programs and treatments proven to produce better outcomes

Facilitating access to historical patient data can also be used to recommend pathways and healthcare settings to appropriate care choices and direct patients to quality care and specialists that offer the best outcomes with full price transparency.

Section D Payors

PA-4. What would be the value to payors of a nationwide provider directory that included FHIR end points and used digital identity credentials?

Payors would directly benefit from a nationwide provider directory that allows them to quickly obtain accurate data that they can use to validate the credentials and licenses of providers. Such a directory would also allow payors to access and validate accurate addresses, phone numbers and fax numbers of providers as well as provider rosters, thereby facilitating the timely, efficient payment of claims.

There are multiple formats for verifying and validating credentialling. These are fragmented and result in inefficiencies. Developing a nationwide provider directory would provide a common process to confirm credentialling status, thereby reducing inefficiencies related to validating clinical and nonclinical credentials. Blockchain technology could be used to certify provider credentialling and licensure through a standard electronic verification format whereby a provider simply reviews and digitally certifies their data through an easy-to-use portal.

State-based credentialing and verification requirements vary tremendously, are tedious and inconsistent, and add costs and inefficiencies to the process of validating and verifying provider credentialling. Having

a national standard to streamline verification would significantly reduce errors and administrative burdens, allowing payors to easily validate credentials and pay claims in a timely manner. Such a directory would also benefit payors and patients if information were also available for patient reviews of their experience with providers, provider quality scores, fee schedules and payment options. This information would allow payors and patients to better evaluate provider options.

Federal and state programs also demand a higher degree of attributable data to drive payment and quality improvement. Many programs are focused on increasing access to advanced primary care models, integrating behavioral health into primary care, and increasing the percentage of dollars spent by payors on primary care and behavioral health services. These programs also leverage value-based payment systems based on the Health Care Payment Learning and Action Network (HCP-LAN) and other models to advance the quality and efficiency of care being delivered to patients across the country. Attributable data is critical to advance any of these goals, especially in capitated arrangements across the US, and a standardized provider identity and directory is a key element towards achieving this goal.

Section E: Technology Vendors, Data Providers and Networks

TD-1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

Creating a national healthcare innovation center of excellence — similar to commercial technology accelerators — and leveraging commercial approaches such as agile development, robust and scalable infrastructure, AI and machine learning to deliver incremental enhancements and tools to patients and caregivers would revolutionize how Americans navigate their healthcare benefits. Too often innovation is slowed by regulations built to provide guardrails for yesterday's technology. Such a public-private partnership can be an opportunity to showcase accelerated innovation and reframe and inform updated safeguards to account for innovative technologies and methodologies. Additionally, patients and caregivers can realize improved outcomes at a lower overall cost. Digital transformation and innovation through a secure platform with standard interfaces, APIs and user-friendly applications will move solutions to the market faster, more accurately and more transparently.

A national healthcare innovation center of excellence that serves as a digital solution accelerator would be a good first step in moving ideas and solutions through a series of gates to assess security, quality, benefit and ease of use for our aging and disadvantaged populations.

From a plan and provider perspective, creating such a center would allow for quicker adoption of tools that would help patients and their caregivers by allowing users to:

- Identify the probability of disease onset
- Provide early intervention strategies
- Show risks in a population through interpretable risk scoring
- Pinpoint with 99% accuracy individuals with higher claim histories to encourage chronic case management or disease management interventions
- Automate standardization and data liquidity
- Streamline administrative processes throughout the stakeholder ecosystem
- Rank risks and prescribe actions, such as ER utilization monitoring, chronic conditions management and high-cost drug utilization management

TD-5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers and payors?

Having a nationwide provider directory of FHIR endpoints can improve access to health information by breaking down barriers and addressing misalignment of provider, patient encounter and claims data across healthcare systems and among stakeholder groups. The directory would facilitate a more complete picture across the continuum of care and inform provider options based on coverage, provider quality, accessibility and type of care.

While a lot of effort has been put forth to streamline the process so that patients and caregivers can easily interact with each other and their care teams, there are several areas that need improvement.

Areas of opportunity include access to accurate and timely data about providers, such as updated addresses, hours of operations, status of associated providers and services, quality scores, patient experience, scheduling tools and fee schedules. These attributes are important for patients and their caregivers to make informed decisions about available care pathways and for payors to quickly adjudicate and pay claims. Including provider and facility access and availability information from EHRs in real-time would help patients seek care from quality providers that have the capacity to service patients in a timely manner, thus preventing deferred care, subsequent hospitalization or costly ER or urgent care visits.

Creating a competitive environment with readily available data through a provider directory interface would also incentivize providers to improve their quality score and patient experience score reflected in reviews, make fee schedules available for patients, provide their location and availability so that a patient can select a provider based on all these criteria, greatly improving timeliness of care delivery and leading to better outcomes.

TD-5. Who should publish such a directory, and should users bear a cost?

CMS should convene stakeholders in a public-private partnership to build on the work that has been ongoing across the country. In its role as convenor and standard setter, CMS could issue guidance for the directory format, design elements, data fields to be updated, timelines for updates and other necessary fields. CMS could also provide seed funding for the initial assessments and a scalable architecture that leverages various efforts in the United States, thereby allowing a level of standardization to occur across directories that are currently in use. Once again, standardizing the format, creating the rules of the road and then leveraging the public-private architecture currently operating across the United States will allow for the right information to be delivered in a timely manner to patients, their caregivers and health care sector players alike.

Given the collective benefit of such a directory across stakeholders, the public-private partnership structure could be leveraged in collaboration with beneficiaries to explore options such as a user fee system based on cost savings and value-based care incentives based on participation to address ongoing sustainment costs.

TD-19. Regarding price transparency implementation:

a. **What are current shortcomings in content, format, delivery and timeliness?**

The MRFs currently available contain core data and a robust roadmap of managed care agreements and information to help patients navigate through the systems. They also create healthy competition among health systems and providers. However, there are elements that could be added to provide clarity and value to healthcare stakeholders, patients and caregivers. These elements include, but are not limited to:

- Validation checks across all data providers' input into the system
- Creation of a universal scoring system to hold organizations accountable for submitting accurate, error-free and timely data.
- Inclusion of summary encounter and billing data to help patients and caregivers understand the complexities of managed care services and dispute unreasonable charges
- Expansion of required healthcare price and coverage data to areas of high volume and significant price variations such as labs, imaging and ambulatory surgery center pricing
- More robust contract reimbursement term definitions that align with different forms of payor and provider payments

There should be two forms of validation for health system and provider inputs:

- Are they abiding by the current formats available?
- Are they providing actionable and reliable information for timely and accurate payment submission?

b. Which workflows would benefit most from functional price transparency?

In accordance with the intent of the 2019 Executive Order and subsequent regulations, these records should support and improve the patient experience during their journey through the health system while reducing unnecessary costs without compromising quality.

To improve the overall patient experience and potential for cost savings, more detailed information and recommendations are needed around:

- Referring patients to an emergency room vs. urgent care facility or next-day office visit
- Comparing the cost of having procedures done in a hospital vs. an outpatient facility or surgical center
- Incentivizing them to choose the lower cost options

Additionally, the intake process at hospitals and other healthcare facilities should include disclosing what services will cost based on a patient's insurance coverage and if any services or providers may be out-of-network, resulting in substantially higher costs.

c. What improvements would be most valuable for patients, providers or payors, including CMS?

With improved data usability and standards in place, patients and healthcare stakeholders will have a clearer view of the quality of care available throughout the delivery systems and will also be afforded detailed information about the cost associated with treatments and services across the continuum of care.

With continued improvement in data integrity and reliability, CMS can realize the goal of delivering an enhanced patient journey with transparency and decision support tools to select the right level of care to manage chronic conditions and episodic events for improved outcomes at a lower overall cost to Medicare, Medicare Advantage and Medicaid programs and participants.

All parties will benefit from reliable, accurate and actionable data, improved decision support and healthcare navigation tools based on common standards and full transparency.

A fully integrated healthcare system that leverages internationally recognized standards related to classification of diseases will assist with diagnosing, coding and tracking diseases and health

conditions. A standardized interface will help to break down barriers created by proprietary systems and will facilitate a fully coordinated national healthcare delivery system.

Leveraging this new system of governance and standards will help to curate personalized and transparent care coordination utilizing the full power of AI and machine learning, empowering patients to choose the best care pathway and providers with the highest quality ratings, lowest overall cost and current capacity to handle patient conditions and needs.

d. What would further motivate solution development?

- More access to data will fuel innovative solutions, drive patients to compliant and quality healthcare systems and support providers with enhanced decision-making capabilities.
- Navigation assistance with full price and quality transparency will incentivize providers to keep developing streamlined applications and curated care experiences for patients and caregivers.
- Clear guidance on transparency requirements based on standard definitions and methodologies can provide certainty in compliance and accelerate innovation in supporting such measures.
- Layering transparency disclosure requirements into existing provider reimbursement incentives, such as star ratings determinations, will motivate further solution development.
- Payors and providers would be motivated to meet established and future standards in order to succeed in the new world of enhanced data usability over time. Including disincentives and penalties for not meeting these standards will further enhance motivation.

Section F: Value-Based Care Organizations

VB-2. How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement and patient engagement be better integrated into APM requirements?

A sound and comprehensive strategic approach that balances innovation with regulatory compliance and patient privacy should be followed when integrating artificial intelligence (AI), population health analytics, risk stratification, care coordination, usability, quality measurement and patient engagement into Alternative Payment Model (APM) requirements. APMs would be greatly improved by incorporating:

- AI-driven decision support tools: AI is the right tool to improve clinical decision-making by analyzing vast datasets to identify trends, predict patient risks and recommend personalized interventions.
- Population health analytics: These help providers track health trends, identify disparities and implement targeted interventions to improve outcomes.
- Risk stratification: Advanced algorithms can categorize patients based on risk levels, allowing for proactive care management and resource allocation.
- Usability: Improved decision support and navigation tools enhance patient experiences, provided they offer both technological and personal assistance such as outreach and manned help centers.
- Quality management: Standard and universal quality measures need to be interoperable, streamlined and agile enough to equip patients to make personalized value assessments and care decisions that are best for them.
- Patient and caregiver engagement: This is a key component to a productive experience for providers, patients and caregivers. It also contributes to making the right informed decisions about treatment and care settings to produce improved experiences and outcomes at a lower overall cost.

Conclusion

In order to transform healthcare systems that support Medicare, Medicare Advantage and Medicaid and empower beneficiaries and their caregivers, we recommend that the following priorities be funded, staffed and implemented:

- A nationwide provider database with accurate and compliant data
- A national healthcare center of innovation excellence accelerating the speed to market of innovative and transformative solutions for patients, caregivers, providers and plans
- A nationwide platform/exchange for patient data that can provide the core building blocks for enhanced care coordination

Harnessing the power of AI within a secure and scalable architecture with the appropriate security protocols would also allow a curated and individualized patient experience without increased cost and would break down the barriers that proprietary systems and technology represent. Creating incentives for adoption of these systems and tools will reward healthcare organizations and providers who are dedicated to reducing costs and adding value by improving a patient's experience throughout their healthcare journey.

Leveraging industry partnerships to build and sustain a robust, scalable architecture for actionable health care data with an optimized search engine can inform AI tools that will provide necessary decision support and navigation tools to curate an individualized healthcare experience for patients. Harnessing technology will provide this benefit without increased costs, while delivering improved outcomes.

Implementing these priorities will also level the healthcare marketplace, driving patients and revenue to quality care organizations that participate fully in the systems above and that provide complete, accurate and actionable data. Implementation of these priorities will also drive improved clinical decision support, healthcare navigation outreach and support for patients and their caregivers.

In summary, by applying these recommendations and principles to Medicare, Medicare Advantage and Medicaid, CMS will set a new standard for America's healthcare systems that empowers patients and caregivers to reduce costs and improve outcomes by driving revenue to quality care with the capacity to meet patient needs and demand.

Claritev stands ready to engage in detailed discussion with CMS, given our over 40 years of proven experience and commitment to public health outcomes coupled with our scale, investment and incorporation of AI technology, speed to deploy transformative solutions, ability to produce reliable data-driven results and modularity to address incremental improvements.

Claritev's corporate values and commitment to improved public health outcomes make us an excellent partner for CMS.

- Due to our operational scale — we identified $24.7 billion in potential savings on $1.7 trillion in claim charges in 2024 alone — Claritev is well positioned to provide CMS with a foundational enterprise system to scale and support outcome-based care.
- We are committed to price transparency, access and equity, providing tools to empower beneficiaries and providers to make informed, cost–effective decisions.
- Our products and healthcare expertise will allow us to serve as a long-term transformational partner for CMS.

A brief introduction to Claritev is included below to substantiate our experience, corporate capability and commitment to helping CMS transform public healthcare.

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About Claritev

Claritev is a healthcare technology, data and insights company that uses tech-enabled solutions to improve affordability, transparency and quality of healthcare. By focusing our purpose-built solutions across the healthcare ecosystem — from payors, employers and patients to providers and third parties — we aim to make healthcare more affordable for all.

Our suite of services supports virtually all types of payors, including employers, brokers and TPAs; medical, supplemental and stop-loss carriers; and benefit administrators and PEOs.

Overview of Claritev Solution Offerings. We offer a range of cost containment and revenue improvement services, including custom networks, out-of-network solutions, surprise billing services, provider payment processing and solutions designed to drive plan design.

At-a-Glance

700+ insurance carriers, plan administrators and other payor customers

2,800 employees

597K claims processed daily

1.4M providers under contract

1B+ claims accessed annually for processing and/or analysis

Network-Based Solutions. Over 1.4 million credentialed providers participate in our healthcare provider networks. Our PHCS Network is the largest independent, nationwide primary preferred provider organization (PPO), and the only national independently contracted primary PPO with NCQA accreditation.

Analytics-Based Solutions. Data-driven, our Analytics-Based Solutions recommend fair reimbursements for out-of-network claims and provide support for NSA compliance. When a patient seeks care from an out-of-network provider, we reduce medical costs by negotiating or determining a fair reimbursement.

Payment and Revenue Integrity Solutions. Our Payment and Revenue Integrity Solutions optimize billing accuracy to lower overall cost of care for payors and members while recommending prices that are fair to the provider community.

Data and Decision Science Solutions. Our Data and Decision Science solutions deliver actionable, digestible data to give providers and payors visibility into their market position, competitive pricing, at-risk members and quality outcomes.

Claritev Payments. Claritev Payments offers clients a single payment solution for settling claims and delivering remittances. The solution minimizes administrative burdens and drives electronic payments to providers, while lowering payment costs and generating revenue or payors.

Industry Awards and Recognition. Claritev has been recognized with numerous awards for our innovative solutions, strong workplace culture and excellence in security. These include:

- Three consecutive Great Place to Work designations
- A 2024 "Achievement in Data and Analytics" Gold Globee® Award
- Three consecutive MedTech Breakthrough Awards for our Ceres®, PlanOptix® and negotiation prioritization solutions
- A 2022 CIO 100 Award
- A 2020 Future Edge 50 Award
- A 2022 "Healthcare Data Solution of the Year" designation

Company History. Claritev was founded in 1980 as a New York-based hospital network and currently operates PHCS – the largest independent provider network in the nation. Prior to 2025, Claritev was known as MultiPlan, Inc. We undertook the rebrand to better represent the important role that we play in the healthcare ecosystem as we leverage our more than 40 years of experience supporting Payors, Providers and Patients. As an industry leader leveraging data insights and automated tools, Claritev helps optimize operations, achieve compliance and navigate the complexities of Medicare, Medicare Advantage and Medicaid programs. The name Claritev reflects the clarity, transparency and innovation behind everything we do.

Structure. Based in McLean, Virginia, Claritev has over 2,800 employees and operates in all 50 states and the District of Columbia and has offices in the following cities:

- Bedford, MA
- New York, NY
- Irving, TX
- Naperville, IL
- Laguna Hills, CA
- McLean, VA